Patient Name	**: Mr. MANGESH DARADE**		Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M		Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408		Report Date	: 13/Jun/2026 06:11PM
Barcode No	**: D3605550**		Client Code	: PUNE616
Ref Doctor	: Dr.SELF		Visit ID	: MPUN355410
Ref Customer	: SELF		Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF HAEMATOLOGY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
HBA1C				
Sample Type : WHOLE BLOOD EDTA				
HbA1c (ngsp)	5.6	%	Non diabetic adults >=18 years <5.7~At risk (Prediabetes) 5.7 - 6.4~Diagnosing Diabetes >= 6.5	HPLC
HbA1c (IFCC)	37.6	mmol/mol		HPLC
Estimated Average Glucose	114.0	mg/dl		Calculated

Interpretation:

As per American Diabetes Association (ADA)	
Reference Group	**HbA1c in %**
Non diabetic adults >=18 years	**<5.7**
At risk (Prediabetes)	**5.7 – 6.4**
Diagnosing Diabetes	**>=6.5**

Note:

1, Since HbA1c reflects long term fluctuation in the blood glucose concentration , a diabetic patient who is recently under good control may still have a high concentration of HbA1c . Converse is true for a diabetic previously under good control but now proply controlled.

2, Target goals of <7.0% may be beneficial in patients with short duration of diabetes , long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes ,limit life expectancy or extensive co-morbid conditions, targeting a goal of <7.0 % may not be appropriate.

Comment :

HBA1c provides an index of average blood glucose levels over the past 8 – 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations.

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**	Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M	Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408	Report Date	: 13/Jun/2026 06:11PM
Barcode No	**: D3605550**	Client Code	: PUNE616
Ref Doctor	: Dr.SELF	Visit ID	: MPUN355410
Ref Customer	: SELF	Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF HAEMATOLOGY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method

Chromatogram Report

TOSOH HLC-723G11 V3.10 1 2026-06-13 17:59:05

ID D3605550
Sample No. 2026061317540203 SL 0006 - 09
Patient ID
Name
Comment

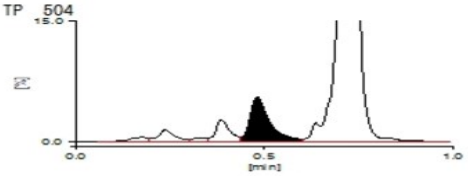
CALIB(N) Y =1.1586X + 0.5297

Name	%	Time	Area
FP			
A1A	0.4	0.17	4.96
A1B	1.0	0.24	12.87
F	0.3	0.33	3.50
LA1C+	1.5	0.39	19.18
SA1C	5.6	0.48	54.69
A0	92.7	0.72	1168.52
H-VAR			
	Total Area		1263.72

HbA1c 5.6 %

HbF 0.3 %

TP 504



13-06-2026 17:59:05 CRL DIAGNOSTICS 1 / 1

CRL DIAGNOSTICS
SHOP NO 2, MODI PLAZA .NEAR KOTAK MAHINDRA BANK
LAXMINARAYAN CHOWK SWARGATE PUNE

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**	Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M	Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408	Report Date	: 13/Jun/2026 05:38PM
Barcode No	**: D3605550**	Client Code	: PUNE616
Ref Doctor	: Dr.SELF	Visit ID	: MPUN355410
Ref Customer	: SELF	Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF HAEMATOLOGY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
COMPLETE BLOOD COUNT (CBC)				
RBC Count	4.74	Millions/cumm	4.5-5.5	Impedance Variation
Haemoglobin (HB)	14.0	g/dl	13.0-17.0	Spectrophotometry
Haematocrit (PCV)	41.6	%	40.0-50.0	Analogical Integration
MCV	87.76	fL	80-100	
MCH	29.54	pg	27.0-32.0	Calculated
MCHC	33.65	g/dL	27.0-48.0	Calculated
RDW-CV	**15.9**	%	11.5-14.0	Calculated
Platelet Count	276	x1000/uL	150-450	Impedance Variation
Total WBC Count	6050	/cumm	4000-10000	Impedance Variation
MPV	**8.40**	%	9.1-11.9	Calculated
PDW	12.80	%	9.0-15.0	Calculated
PCT	0.23	%	0.18-0.39	Calculated
Differential Leucocyte Count				
Neutrophil	52.5	%	40.0-80.0	flow cytometry/manual
Lymphocyte	35.2	%	20.0-40.0	flow cytometry/manual
Monocytes	6.2	%	2-10	flow cytometry/manual
Eosinophils	4.6	%	01-06	Flow cytometry/manual
Basophils	1.5	%	0-2	Flow cytometry/manual
Absolute Neutrophils	3.18	1000/uL	2.00-7.00	
Absolute Lymphocytes	2.13	1000/µL	1.00-3.00	
Absolute Monocytes	0.38	1000/µL	0.20-1.00	
Absolute Eosinophils	0.28	1000/µL	0.02-0.50	
Absolute Basophils	0.09	1000/µL	0.02-0.10	
Neutrophil-Lymphocyte Ratio	1.49			Calculated
Lymphocyte-Monocyte Ratio	6			Calculated
Platelet-Lymphocyte Ratio	8			Calculated

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**	Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M	Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408	Report Date	: 13/Jun/2026 06:50PM
Barcode No	**: D3605550**	Client Code	: PUNE616
Ref Doctor	: Dr.SELF	Visit ID	: MPUN355410
Ref Customer	: SELF	Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF HAEMATOLOGY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
Erythrocyte Sedimentation Rate (ESR)	12	mm/h	0-20	Westergren

Patient Name	**: Mr. MANGESH DARADE**	Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M	Specimen Received ON	: 13/Jun/2026 04:29PM
UHID/MR No	: APUN.0000355408	Report Date	: 13/Jun/2026 05:46PM
Barcode No	**: D3605549**	Client Code	: PUNE616
Ref Doctor	: Dr.SELF	Visit ID	: MPUN355410
Ref Customer	: SELF	Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF BIOCHEMISTRY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
GLUCOSE FASTING				
Sample Type : Sod.Fluoride - F				
Glucose Fasting	79.10	mg/dl	70.0 - 110.0	GOD-POD

Interpretation (In accordance with the American diabetes association guidelines):

- A fasting plasma glucose level below 110 mg/dL is considered normal.
- A fasting plasma glucose level between 100-126 mg/dL is considered as glucose intolerant or pre diabetic. A fasting and post-prandial blood sugar test (after consumption of 75 gm of glucose) is recommended for all such patients.
- A fasting plasma glucose level of above 126 mg/dL is highly suggestive of a diabetic state. A repeat fasting test is strongly recommended for all such patients. A fasting plasma glucose level in excess of 126 mg/dL on both the occasions is confirmatory of a diabetic state.

This report has been validated by:

DR. SHIRISH POPHALIKAR M.B.B.S , M.D. (PATH) CONSULTANT PATHOLOGIST REGD. NO. 2002/01/0139	DR. EKTA PANDEY M.B.B.S , M.D. (PATH) CONSULTANT PATHOLOGIST REGD. NO. 2013030621	DR. ABHINAV WALIA M.B.B.S , M.D. (PATH) CONSULTANT PATHOLOGIST REGD. NO. 2015094877	DR. SUSHAMA T ROKADE M.B.B.S , M.D. (MICRO) CONSULTANT MICROBIOLOGIST REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**	Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M	Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408	Report Date	: 13/Jun/2026 07:28PM
Barcode No	**: D3605552**	Client Code	: PUNE616
Ref Doctor	: Dr.SELF	Visit ID	: MPUN355410
Ref Customer	: SELF	Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF BIOCHEMISTRY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
Calcium	9.15	mg/dL	8.6-10.2	NM-BAPTA

DESCRIPTION

About 50% of the calcium present in circulation is free (also known as ionized calcium); 40% of serum calcium is bound to proteins, especially albumin (80%) and, secondary, to globulins (20%); and about 10% exists as various small diffusible inorganic and organic anions (eg, bicarbonate, lactate, citrate).Heart and skeletal muscle contractility are affected by calcium ions; in addition, calcium ions are vital to nervous system function and are associated with blood clotting and bone mineralization.The concentration of serum calcium is tightly regulated by parathyroid hormone (PTH) and 1,25-hydroxy vitamin D.

I**NTERPRETATION-**

Serum calcium is decreased (*hypocalcemia*) in the following conditions:

Hypoparathyroidism , Vitamin D deficiency ,Chronic renal diseases ,Pseudohypoparathyroidism,
Magnesium deficiency (PTH glandular release is magnesium-dependent), Hyperphosphatemia,
Massive transfusion, Hypoalbuminemia, Severe calcium dietary deficiency and Severe pancreatitis (calcium saponification)

Serum calcium is increased(*Hypercalcemia*) in the following conditions:

Hyperparathyroidism ,Vitamin D excess, Milk-alkali syndrome, Multiple myeloma, owing to bone lesions, Paget disease of bone with prolonged immobilization, Sarcoidosis, Familial hypocalciuria hypercalcemia,Vitamin A intoxication,Thyrotoxicosis and Addison disease

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**	Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M	Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408	Report Date	: 13/Jun/2026 07:28PM
Barcode No	**: D3605552**	Client Code	: PUNE616
Ref Doctor	: Dr.SELF	Visit ID	: MPUN355410
Ref Customer	: SELF	Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF BIOCHEMISTRY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
EGFR (ESTIMATED GLOMERULAR FILTRATION RATE)				
Creatinine	0.92	mg/dL	0.70-1.40	Jaffe Kinetic
Blood Urea Nitrogen (BUN)	9.23	mg/dL	6-20	spectrophotometry
Albumin (Serum)	4.16	g/dL	3.5-5.5	Bromo Cresol Green (BCG)
EGFR By MDRD	105.06	mL/min/1.73 m2		Spectrophotometric - Calculated

COMMENT-The Kidney Disease Improving Global Outcomes (KDIGO) guideline defines CKD by the presence of glomerular filtration rate (GFR) <60 mL/min/1.73m2 for >3 months and/or evidence of kidney damage (eg, structural abnormalities, histologic abnormalities, albuminuria, urinary sediment abnormalities, renal tubular disorders, and/or history of kidney transplantation) for >3months.2 Thus, monitoring should include tests for GFR, albuminuria, and urine sediment.

CLINICAL USE-

• Detect chronic kidney disease (CKD) in adults.

• Monitor CKD therapy and/or progression in adults.

Interpretation of eGFR Values	
eGFR (mL/min/1.73m^2)	**Interpretation**
90	Normal
60-89	Mild decrease
45-59	Mild to moderate decrease
30-44	Moderate to severe decrease
15-29	Severe decrease
<15	Kidney failure

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**		Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M		Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408		Report Date	: 13/Jun/2026 07:28PM
Barcode No	**: D3605552**		Client Code	: PUNE616
Ref Doctor	: Dr.SELF		Visit ID	: MPUN355410
Ref Customer	: SELF		Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF BIOCHEMISTRY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
LIVER FUNCTION TEST (LFT)-EXTENDED				
Sample Type : SERUM				
Bilirubin Total	0.71	mg/dl	<1.1	Diazotized Sulfanilic
Bilirubin Direct	0.14	mg/dL	0-0.3	
Bilirubin Indirect	0.57	mg/dl	0.30-1.00	Calculated
SGOT (AST)	25.54	U/L	<31.0	IFCC without pyridoxal phosphate
SGPT (ALT)	28.21	U/L	<33.0	IFCC without pyridoxal phosphate
Alkaline Phosphatase (ALP)	95.45	U/L	40-129	IFCC Standardization
Gamma Glutamyl Transferase (GGT)	22.97	U/L	15-60	L-Gamma-glutamyl-3-carboxy-4-nitroanilide Substrate
Protein Total	7.16	g/dL	6.6-8.7	Biuret
Albumin (Serum)	4.16	g/dL	3.5-5.5	Bromo Cresol Green (BCG)
Globulin	3	g/dL	2.50-3.50	Calculated
A/G Ratio	**1.39**		1.5-2.5	Calculated

Interpretation:- Liver blood tests, or liver function tests, are used to detect and diagnose disease or inflammation of the liver. Elevated aminotransferase (ALT, AST) levels are measured as well as alkaline phosphatase, albumin, and bilirubin. Some diseases that cause abnormal levels of ALT and AST include hepatitis A, B, and C, cirrhosis, iron overload, and Tylenol liver damage. Medications also cause elevated liver enzymes. There are less common conditions and diseases that also cause elevated liver enzyme levels.: Liver blood tests, or liver function tests, are used to detect and diagnose disease or inflammation of the liver. Elevated aminotransferase (ALT, AST) levels are measured as well as alkaline phosphatase, albumin, and bilirubin. Some diseases that cause abnormal levels of ALT and AST include hepatitis A, B, and C, cirrhosis, iron overload, and Tylenol liver damage. Medications also cause elevated liver enzymes.There are less common conditions and diseases that also cause elevated liver enzyme levels.

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**	Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M	Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408	Report Date	: 13/Jun/2026 07:28PM
Barcode No	**: D3605552**	Client Code	: PUNE616
Ref Doctor	: Dr.SELF	Visit ID	: MPUN355410
Ref Customer	: SELF	Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF BIOCHEMISTRY

PUNE CRL PACKAGE

LIPID PROFILE BASIC

Sample Type : SERUM

Test Name	Result	Unit	Bio. Ref. Range	Method
Total Cholesterol	**283.95**	mg/dL	Desirable - 200, Borderline high - 200-239, High - ≥ 240	CHO-POD
Triglyceride	**155.29**	mg/dL	0.0-150 :Normal 151-199:Border Line >=200 :High 200.0-499.0 High ~> 500 Very High	GPO-POD
HDL Cholesterol	**64.50**	mg/dL	40-60	Direct (PVS/PEGME precipitation & Trinder reaction)
Non HDL Cholesterol	**219.45**	mg/dL	< 130 mg/dL	Calculated
VLDL Cholesterol	**31.1**	mg/dL	2.00-30.00	Calculated
LDL Cholesterol	**188.39**	mg/dL	0-130 :Normal~131-155:Borderline~>=160 :High	Direct (PVS/PEGME precipitation & Trinder reaction)
Cholesterol/HDL Ratio	**4.40**	Ratio	<4.00	Calculated
LDL / HDL Cholestrol Ratio	2.92	Ratio	<3.50	Calculated
HDL/LDL Cholesterol Ratio	0.34	Ratio	<3.50	Calculated

Cholesterol Level	**mg/dL**
Desirable	200
Borderline High	200 - 239
High	≥ 240

Risk Modifiers As per ASCVD

PARAMETRS	**mg/dL**
HDL	<40 - low
	>60 - high
LDL	<100 optimal
TRIGLYCERIDE LEVELS	< 150 for fasting
	< 175 for Non fasting

Treatments Goal as per LAI 2023

ASCVD RISK CATEGORY	**TREATMENT GOAL**	
	LDL-C in mg/dL Primary Target	**NonHDL-C in mg/dL CO-Primary Target**
LOW	<100	**<130**
MODERATE	<100	**<130**
HIGH	<70	**<100**
VERY HIGH	<50	**<80**
EXTREME (A)	<50 or <30	**<80 or <60**
EXTREME (B)	<30	**<60**

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**	Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M	Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408	Report Date	: 13/Jun/2026 07:28PM
Barcode No	**: D3605552**	Client Code	: PUNE616
Ref Doctor	: Dr.SELF	Visit ID	: MPUN355410
Ref Customer	: SELF	Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF BIOCHEMISTRY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
IRON PROFILE BASIC				
Iron, Serum	103.43	ug/dL	59-158	Colorimetric
Total Iron Binding Capacity-(TIBC)	**405.6**	ug/dL	250-400	Spectro-photometry
UIBC-SERUM	302.17	ug/dL	110-370	Direct Determination with Ferrozinc
Transferrin Saturation	25.50	%	16-50	Calculated

Total iron-binding capacity

The test measures the extent to which iron-binding sites in the serum can be saturated. Because the iron-binding sites in the serum are almost entirely dependent on circulating transferrin, this is really an indirect measurement of the amount of transferrin in the blood.

Taken together with serum iron and percent transferrin saturation clinicians usually perform this test when they are concerned about anemia, iron deficiency or iron deficiency anemia. However, because the liver produces transferrin, liver function must be considered when performing this test. It can also be an indirect test of liver function, but is rarely used for this purpose

Transferrin Saturation

1g of transferrin can carry 1.43g of iron. Normally, iron saturation of transferrin (transferrin saturation) is between 10% and 50%. Because of its short half-life, transferrin values decrease more quickly in protein malnutrition states and should be taken into consideration while evaluating iron-deficiency states

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**	Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M	Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408	Report Date	: 13/Jun/2026 07:28PM
Barcode No	**: D3605552**	Client Code	: PUNE616
Ref Doctor	: Dr.SELF	Visit ID	: MPUN355410
Ref Customer	: SELF	Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF BIOCHEMISTRY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
CRP QUANTITATIVE:-				
CRP(C-Reactive Protein)-Quantitative:-	3.41	mg/L	<5.00	Turbidimetric Immunoassay

INTERPRETATION-

CRP is an acute phase reactant which is used in inflammatory disorders for monitoring course and effect of therapy. It is most useful as an indicator of activity in Rheumatoid arthritis, Rheumatic fever, tissue injury or necrosis and infections. As compared to ESR, CRP shows an earlier rise in inflammatory disorders which begins in 4-6 hrs, the intensity of the rise being higher than ESR and the recovery being earlier than ESR. Unlike ESR, CRP levels are not influenced by hematologic conditions like Anemia, Polycythemia etc.

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**	Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M	Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408	Report Date	: 13/Jun/2026 07:28PM
Barcode No	**: D3605552**	Client Code	: PUNE616
Ref Doctor	: Dr.SELF	Visit ID	: MPUN355410
Ref Customer	: SELF	Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF BIOCHEMISTRY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
Kidney Function Test EXTENDED				
Urea	19.76	mg/dL	17.0-43.0	Urease UV
Creatinine	0.92	mg/dL	0.70-1.40	Jaffe Kinetic
Calcium	9.15	mg/dL	8.6-10.2	NM-BAPTA
Uric Acid	4.80	mg/dl	4.40-7.60	Spectro-photometry
Phosphorus	**5.19**	mg/dL	2.50-5.00	Ammonium molybdate UV
Sodium (NA+)	142.00	mmol/L	135.0-145.0	Ion Selective Electrode
Potassium (K+)	3.68	mmol/L	3.50-5.50	Ion Selective Electrode
Chloride	102.00	mmol/L	98.0-109.0	Ion Selective Electrode
Blood Urea Nitrogen (BUN)	9.23	mg/dL	6-20	spectrophotometry
Bun / Creatinine Ratio	10.03	Ratio	0.0-23.0	Calculated
Urea / Creatinine Ratio	21.48	Ratio	20-35	Calculated

Interpretation:- Kidney blood tests, or Kidney function tests, are used to detect and diagnose disease of the Kidney.

The higher the blood levels of urea and creatinine, the less well the kidneys are working.
The level of creatinine is usually used as a marker as to the severity of kidney failure. (Creatinine in itself is not harmful, but a high level indicates that the kidneys are not working properly. So, many other waste products will not be cleared out of the bloodstream.) You normally need treatment with dialysis if the level of creatinine goes higher than a certain value.
Dehydration can also be a come for increases in urea level.
Before and after starting treatment with certain medicines. Some medicines occasionally cause kidney damage (Nephrotoxic Drug) as a side-effect. Therefore, kidney function is often checked before and after starting treatment with certain medicines.

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**		Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M		Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408		Report Date	: 13/Jun/2026 09:15PM
Barcode No	**: D3605552**		Client Code	: PUNE616
Ref Doctor	: Dr.SELF		Visit ID	: MPUN355410
Ref Customer	: SELF		Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF IMMUNOASSAY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
THYROID PROFILE				
Sample Type : SERUM				
Triiodothyronine Total (T3)	0.93	ng/mL	0.70-2.04	Chemiluminescence Immunoassay (CLIA)
Thyroxine Total (T4)	8.6	ug/dL	5.48-14.28	
TSH (4th Generation)	**37.608**	uIU/mL	0.40-4.20	Chemiluminescence Immunoassay (CLIA)

PREGNANCY	REFERENCE RANGE for TSH IN uIU/mL (As per American Thyroid Association.)
1st Trimester	0.10-2.50 uIU/mL
2nd Trimester	0.20-3.00 uIU/mL
3rd Trimester	0.30-3.00 uIU/mL

INTERPRETATION-

1. Primary hyperthyroidism is accompanied by elevated serum T3 & T4 values along with depressed TSH level.
2 .Primary hypothyroidism is accompanied by depressed serum T3 and T4 values & elevated serum TSH levels.
3. Normal T4 levels accompanied by high T3 levels and low TSH are seen in patients with T3 thyrotoxicosis.
4. Normal or low T3 & high T4 levels indicate T4 thyrotoxicosis (problem is conversion of T4 to T3)
5. Normal T3 & T4 along with low TSH indicate mild / subclinical HYPERTHYROIDISM .
6. Normal T3 & low T4 along with high TSH is seen in HYPOTHYROIDISM .
7. Normal T3 & T4 levels with high TSH indicate Mild / Subclinical HYPOTHYROIDISM .
8. Slightly elevated T3 levels may be found in pregnancy and in estrogen therapy while depressed levels may be encountered in severe illness , malnutrition , renal failure and during therapy with drugs like propanolol.
9. Although elevated TSH levels are nearly always indicative of primary hypothroidism . rarely they can result from TSH secreting pituitary tumours (seconday hyperthyroidism)

TSH IS DONE BY ULTRASENSITIVE 4th GENERATION CHEMIFLEX ASSAY

COMMENTS:

Assay results should be interpreted in context to the clinical condition and associated results of other investigations. Previous treatment with corticosteroid therapy may result in lower TSH levels while thyroid hormone levels are normal. Note:

TSH levels may fluctuate based on few factors such as pregnancy, illness and age. Also, time of sample collection, technologies used to analyze the test, usage of certain drugs. Diet may have impact on TSH levels. TSH may show around 50% variation even when done at different times of day due to its association with circadian rhythm.

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**	Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M	Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408	Report Date	: 13/Jun/2026 09:15PM
Barcode No	**: D3605552**	Client Code	: PUNE616
Ref Doctor	: Dr.SELF	Visit ID	: MPUN355410
Ref Customer	: SELF	Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF IMMUNOASSAY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
VITAMIN B12				
Sample Type : SERUM				
Vitamin B12 Level	330	pg/mL	220-914	Chemiluminescence Immunoassay(CLIA)

Comments

Vitamin B_{12} along with folate is essential for DNA synthesis and myelin formation. Vitamin B_{12} deficiency can be because of nutritional deficiency, malabsorption and other gastrointestinal causes. The test is ordered primarily to help diagnose the cause of macrocytic/ megaloblastic anemia.

Decreased levels are seen in:	Increased levels are seen in:
anaemia, normal near term pregnancy, vegetarianism, partial gastrectomy/ ileal damage, celiac disease, with oral contraceptive use, parasitic competition, pancreatic deficiency, treated epilepsy, smoking, hemodialysis and advancing age	renal failure, hepatocelluar disorders, myeloproliferative disorders and at times with excess supplementation of vitamins pills

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE

Patient Name	**: Mr. MANGESH DARADE**	Specimen Drawn ON	: 13/Jun/2026 04:29PM
Age/Gender	: 27 YRS /M	Specimen Received ON	: 13/Jun/2026 04:32PM
UHID/MR No	: APUN.0000355408	Report Date	: 13/Jun/2026 08:25PM
Barcode No	**: D3605552**	Client Code	: PUNE616
Ref Doctor	: Dr.SELF	Visit ID	: MPUN355410
Ref Customer	: SELF	Client Name	: UNITY CLINICAL LAB

DEPARTMENT OF IMMUNOASSAY

PUNE CRL PACKAGE

Test Name	Result	Unit	Bio. Ref. Range	Method
VITAMIN D3 25-HYDROXY				
Sample Type : SERUM				
Vitamin D, 25 Hydroxy	**15.8**	ng/mL	Deficiency - <20~Insufficiency - 20-30~Sufficiency - 30 - 100~Toxicity- >100	Chemiluminescence Immunoassay (CLIA)

Lower-than-normal levels suggest a vitamin D deficiency. This condition can result from Lack of exposure to sunlight,Lack of adequate vitamin D in the diet, Liver and kidney diseases and Malabsorption. A vitamin D deficiency may lead to: *Low blood calcium levels (hypocalcaemia) *Thin or weak bones (rickets, osteoporosis and osteomalacia) *High levels of parathyroid hormone (secondary hyperparathyroidism) Total 25-hydroxyvitamin D (D2 + D3) is the correct measure of Vitamin D status.Higher-than-normal levels suggest excess vitamin D, a condition called hypervitaminosis D. It is usually caused by vitamin D in the form of doctor-prescribed dietary supplements. 95% of serum vitamin D is Vit D3. D2 is only received from supplements.

***** End Of Report *****

This report has been validated by:

DR. SHIRISH POPHALIKAR
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2002/01/0139

DR. EKTA PANDEY
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2013030621

DR. ABHINAV WALIA
M.B.B.S , M.D. (PATH)
CONSULTANT PATHOLOGIST
REGD. NO. 2015094877

DR. SUSHAMA T ROKADE
M.B.B.S , M.D. (MICRO)
CONSULTANT MICROBIOLOGIST
REGD. NO. 2117

QR CODE